✻ WHEN FOOD STUFF IS BRAIN STUFF

You found the chair.

A soft place to start with ADHD, eating, and the body — for when the usual advice never quite fit.



pull up a chair.

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✱ WHY THIS GUIDE

Often, it was never just about willpower.

If you're neurodivergent, eating can be tangled up with how your brain works — not a character flaw, and not something you failed at. Many ADHDers find that hunger and fullness signals run quiet or arrive late, that food becomes the nearest place to put a big feeling, or that a "good eating day" depends entirely on whether the day itself felt regulated.

Most eating advice was built for a steady, predictable signal. ADHD brains don't always run steady — so "just listen to your body" and "everything in moderation" can land as a rulebook that was never built for you.

This guide is a gentle place to begin. It won't tell you what to eat, and it isn't a diagnosis. It's a few ways of understanding the overlap, and some soft starting points — so the next step, if and when you want one, feels a little more possible.

✱ WHO IT'S FOR

Anyone curious about where ADHD, eating and body image meet — whether you have a diagnosis, suspect one, or just know the usual advice hasn't fit. You don't need all the words to start.

✱ WHAT THIS CAN LOOK LIKE

Food stuff that's often brain stuff.

A few of the patterns we hold in the room. You might recognise one, or all of them — or none, and that's okay too.

THE SIGNAL THAT RUNS QUIET

When hunger, fullness and "I need to eat" are hard to read, eating becomes guesswork — or gets forgotten until it's urgent. This is interoception, and it's a skill that can be gently noticed and rebuilt, not a flaw to fix.

WHEN A COMMENT FOLLOWS YOU TO THE TABLE

For sensitive nervous systems, a small remark about food or body can flip a whole day. That's not you being "too sensitive" — it's a nervous system turned up loud, with food the nearest place to put it.

THE STIMULANT-MEDS CRASH

Appetite quiet all day, then arriving all at once after the meds wear off. It's a signal that switched off and came back loud — not a discipline problem.

EATING THAT NEEDS SCAFFOLDING

When executive function is stretched, regular eating falls through the cracks. The aim isn't more willpower — it's supports that fit a real, ADHD day, not an idealised one.

✱ A FEW GENTLE STARTING POINTS

Small, kind, and built for your brain.

None of these are rules. They're things some people find help — try one, leave the rest. The goal is "good enough and kind," not perfect.

01 Outsource the signal

If hunger is hard to read, let something external carry it — a gentle phone alarm, eating anchored to things you already do, or a "I haven't eaten in a while, that counts" rule of thumb.

02 Lower the bar on purpose

A "good enough" snack you'll actually eat beats an ideal meal that never happens. Pre-made, easy, repetitive and safe foods are a strategy, not a failure.

03 Be kind to the after-meds crash

If appetite disappears on stimulant meds and returns at night, plan for it gently rather than fighting it — keep easy food nearby for when the signal comes back loud.

04 Notice without scoring

You can pay attention to eating without grading it good or bad. Curiosity ("huh, that's interesting") settles the nervous system in a way judgement never does.

05 Let it be a team effort

You don't have to hold this alone. A GP, dietitian, or therapist who gets the neurodivergent brain can share the load — that's what the next page is for.

✱ IF YOU'D LIKE SUPPORT

There's no bar you have to clear first.

You don't need to be "sick enough," or sure it's "an eating disorder," to reach out. If eating, body image or the way your brain and food meet has become hard to carry, that's reason enough.

Support might be ongoing therapy, a single conversation, or simply somewhere to start making sense of things. It can be by telehealth anywhere in Australia, from wherever suits you. Medicare rebates may be available for eligible clients with the right referral or plan — your GP can help you find the pathway that fits.

✱ A NOTE ON SAFETY

This guide is general information — not therapy, and not a diagnosis. It isn't a crisis service. If you or someone else is in immediate danger, call **000**.

For 24/7 support in Australia:
Lifeline 13 11 14 · **13YARN** 13 92 76 (Aboriginal & Torres Strait Islander)
Kids Helpline 1800 55 1800 · **Butterfly Foundation** 1800 33 4673 (eating disorders)

✱ BEGIN GENTLY

Pull up a chair.

we kept one for you.

When you're ready — whether that's now, or a long way off — there's a warm, credentialed, neurodivergent-affirming room here for you. No pressure to have it all worked out first.

How to begin

Book or ask about fit: bodybelongingclinic.com.au

Email: admin@bodybelongingclinic.com.au

Instagram: @bodybelongingclinic

Lauren Lynch (she/her) is an Accredited Mental Health Social Worker (AASW) and ANZAED Credentialed Eating Disorder Clinician, offering support in Boorloo (Perth) and by telehealth across Australia.

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